

This situation will come about very soon if proper steps are not taken.

More than 75 per cent of the healthcare delivery is undertaken by the private sector. India may have the largest number of beds but these beds are utilised mainly for medical treatment, not for intervention. What a patient today requires is an intervention in his body, which is offered primarily by the private sector. This is where the second step comes in. Policymakers should understand that the poor fall sick as much as the rich do and any attempt to earn money by charging high electricity bills and various taxes to a hospital is essentially making healthcare more expensive and less accessible to the poor. So, the government should not look at the healthcare industry as an avenue for tax collection. When this stops, the cost of healthcare will decrease by 25 per cent making it affordable for everyone. Today, hospitals pay a higher charge for electricity compared to that for households because this industry is considered the same as any other industry.

Third, the government should become a health insurance provider rather than a healthcare provider. Five years ago when Karnataka's state government launched the Yeshaswini Micro Health Insurance programme through the cooperative society network, nobody believed that with Rs 5 a month one could do a heart operation. Today more than three million farmers are benefiting from this scheme. On similar lines, the Andhra Pradesh government launched the Arogya Shree Yojana which is helping nearly 80 per cent of the state's population of BPL cardholders. As time passes, it will become extremely difficult for the government to manage large-scale specialty hospitals. It is going to be more complicated than managing a software company by controlling the policies of healthcare delivery and expecting high standards. The government can become a regulatory body and manage a network of health insurance by creating an infrastructure where every citizen of the country can contribute a small amount of money for health insurance. One can charge one's insurance card by going to the post office or ATM. We strongly believe that all across the world healthcare delivery cost will be dissociated from the taxpayer's

money as well as the family savings. There will be a mechanism for everyone to follow, starting from newborn babies to a geriatric group as a monthly contribution. By the sheer size of this contribution, everyone will have equal access to the healthcare delivery system.

The fourth step is that every city's plan should allocate large spaces for constructing large hospitals and the building norms for the hospital should be radically different than that of a commercial development area. This land should be allotted at a discounted price in order to promote constructing of large hospital complexes in cities all over India.

Fifth, the Reserve Bank of India should treat the healthcare industry as a priority sector and arrange financial assistance for a large-scale project. Currently, financing by a bank is at high interest and with a repayment schedule of five years, which is very difficult for a hospital to comply with since the industry requires a huge capital outlet. Ideally, hospitals should get funding with low interest rates to be repaid over a period of 10 years rather than five years. It can be a ballooning type of repayment in which a large sum comes in the last three to four years to the banks.

The last step is the extension of the tax exemption status for new projects over 100 beds in major cities. Currently, new hospitals with over 100 beds in Tier-II cities are exempted from income tax in the first five years. It should be extended for all new projects across the

country so that the number of beds available in India will go up significantly by reducing the cost of healthcare.

The new government should create jobs for semi-skilled and unskilled youngsters coming from rural India. Unless the industry is sufficiently incentivised in terms of tax concession, land at a discounted price and launching of large-scale micro-health insurance programmes, none of the efforts will give expected results. Today, citizens' illnesses are a great source of revenue—almost 25 per cent of the hospital billing goes back to government in various forms of taxes. More sick people translate into high revenue. But that should not be the case. The government should support the health sector and make healthcare affordable for all. ■



OVER 75 PER CENT OF HEALTHCARE INSTITUTIONS ARE RUN PRIVATELY, MAKING TREATMENT EXPENSIVE FOR THE POOR.